National Cancer Control Programme (NCCP) 00691 · clinician draft

FOLFIRINOX followed by chemoradiotherapy · rectal cancer

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

FOLFIRINOX induction: six 14-day cycles

Active medicines: Oxaliplatin + Folinic acid + Irinotecan + Fluorouracil

Emetogenic risk: High

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 before chemotherapy Check interactions; national dexamethasone doses below already account for this combination.
Aprepitant	80 mg orally once daily	Days 2 and 3 Do not duplicate another NK1 antagonist.
Ondansetron	16 mg orally once	Day 1 before chemotherapy Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	12 mg orally once	Day 1 before chemotherapy Reconcile all steroids; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2, 3 and 4 Re-anchor to the actual treatment day if delayed.
Olanzapine	5 mg orally once daily	Days 1, 2, 3 and 4 Review sedation, falls, glucose and interactions.
Metoclopramide	10 mg orally up to three times daily as required	Subsequent-day rescue; prescriber confirms duration Review renal function and neurological contraindications. Concomitant olanzapine increases extrapyramidal adverse-effect risk; reconcile rescue medicines.
Atropine sulphate	250 micrograms subcutaneously	Before irinotecan if prescribed Avoid in glaucoma; use caution with asthma. This treats acute cholinergic effects, not delayed diarrhoea.

Additional phase-specific support

- This chemotherapy combination has a high risk of sickness without prevention. Follow the prescribed prevention and rescue medicines. Call if vomiting prevents drinking; do not duplicate steroids or anti-sickness doses.
- Growth-factor injections depend on your risk, blood counts and previous complications. If prescribed, confirm their timing in relation to chemotherapy and pump removal; do not assume everyone receives them.
- Atropine is normally prescribed before irinotecan for early cramps, sweating or diarrhoea. It is avoided with glaucoma; tell the team about asthma as well. At home use the separately prescribed delayed-diarrhoea instructions. This can differ from standard over-the-counter dosing. Keep the medicine and urgent-contact plan available.
- Avoid cold drinks, cold air and cold objects if they trigger symptoms. Do not use ice chips during oxaliplatin treatment. Ask for non-cold mouth-care advice. New breathing difficulty must still be assessed rather than assumed to be a cold reaction.
- Capecitabine alone has a lower sickness risk than the induction combination, so the supportive prescription changes with the phase. Ask about pelvic skin care, bowel and bladder symptoms, diet and radiotherapy preparation.
- Irinotecan delayed-diarrhoea rescue must be supplied as a written prescription: loperamide 4 mg initially, then 2 mg every 2 hours until 12 hours after the last liquid stool, for no longer than 48 hours at this high-dose schedule. Fever, dehydration, severe pain or persistent diarrhoea needs urgent oncology assessment. Do not carry this schedule into capecitabine treatment without a new prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00691 v4 · NCCP NCIS antiemetics V8: high-risk non-AC row; multiday support reviewed by prescriber

Sources checked 2026-11-04 · SACTCheck v0.84.33 · Regimen-level drafts; prescriber and oncology-pharmacy verification required